NCCP 00253 · VERSION 6

Tamoxifen · daily hormone therapy

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or G-CSF order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Report abnormal vaginal bleeding, clot symptoms and visual changes promptly. Discuss hot flushes, sleep and sexual symptoms early.
- History of significant thromboembolic disease is an NCCP exclusion. Severe hepatic impairment is not recommended. Interactions, including potent CYP2D6 inhibitors, need medication review.
- Assess personal fracture and menopausal risks. Tamoxifen has a different effect on bone from aromatase inhibitors; do not automatically copy an AI bone-density schedule.
- Assess persistent symptoms and prescribe individually; no automatic prophylactic antiemetic, steroid or G-CSF.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00253 v6 · Monitoring and treatment map

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending